جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : حمديه عبدالرحمن دويب حسن
المحافظة : القاهره **الحي / المدينة :** مصر القديمه **العنوان :** 0
رقم البطاقة : 27406222402504 **رقم التليفون :** 01129849402 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 22-06-1974 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: حمديه عبدالرحمن دويب حسن
National ID: 27406222402504

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


٢٧٤٠٦٢٢٢٤٠٢٥٠٤
٢٠٢٥/٠٥
رية منزل
أنثى
سلمة
متزوجة
حسن كامل سعد مهدى
البطاقة سارية حتى ٢٠٣٢/٠٥/١٩


جمهورية مصر العربية
بطاقة تحقيق الشخصية
حمديه
عبدالرحمن دويب حسن
قرية سانقولا
مركز بنى مزار - المنيا
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مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

06/30/2026

MDT FORM

Name:	Hamdya Abd Elrahman Doleb	**MRN:**	4227
Age:	52 Y	**ID:**	27406222402504
Menopausal Sta	hysterectomy but no oopherectomy for fibroids	**Phone:**	01150995693
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	

Clinical Data:

BX: IDC

ER: 8/8 **PR:** 8/8 **HER2:** 3+ **KI67:**

Sonomammography:

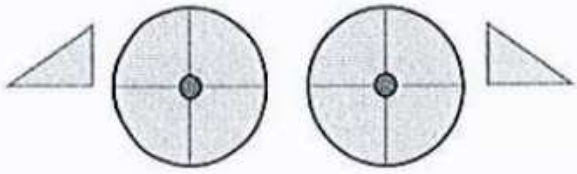
Mets Workup:
12/2024: MBC, received phesgo docetaxel 4 cycles then maintenance phesgo - femara
7/25 PD, shifted to xeloda - tykerb

History:
CT CAP10/5/26: PD of Pulmonary nodules
Bilateral SM: Left breast Mass 4.7 x 3 cm, left axillary mass: 2.7 x 2.2 cm + multiple axillary LN
O/E: Left breast Mass causing ulcer in the skin and opening.
For MDT about further ttt (patient is refusing further chemotherapy) and seeking palliative surgery to the left breast

Recommendations:
Conider TDM-a in ngo + it MRM
LT MRM FEASIBLE BY EXAM DR NBRAWAY 27/6

MDT Outcome & Plan
LT MRM
+TDM1 in NGO.

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د / خالد عبدالعزيز	د / محمد البيروني				

MDT Form
[[DOCTYPE.MDT_FORM]]

Name: حمديه عبد الرحمن دويب	
MR. : 4227	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : تاريخ وساعة الخروج :

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 52-year-old individual with a history of hysterectomy performed for fibroids, without oophorectomy, and therefore is not classified as menopausal. The clinical course includes a diagnosis of metastatic breast cancer in December 2024, initially treated with four cycles of Phesgo plus docetaxel followed by maintenance Phesgo and letrozole (Femara). Disease progression was documented in July 2025, prompting a switch to capecitabine (Xeloda) and lapatinib (Tykerb).

Imaging demonstrates progressive disease with pulmonary nodules noted on a CT chest-abdomen-pelvis performed on 10 May 2026. Bilateral sonomammography reveals a left breast mass measuring 4.7 × 3 cm, an adjacent left axillary mass measuring 2.7 × 2.2 cm, and multiple enlarged axillary lymph nodes. Physical examination confirms that the left breast mass has ulcerated the overlying skin, creating an open wound.

The referring team recommends palliative left-side mastectomy with the goal of achieving free margins, including free deep margins, consistent with a T4b classification. The multidisciplinary team concurs with this plan and will proceed with palliative left-side mastectomy to obtain clear surgical margins.

تحريرا في ,

التاريخ : 23-09-2026 الوقت :

الطبيب المعالج :

يعتمد ,

t Name	: الأستاذه / حمديه عبدالرحمن دويب حسن	Request No.	: 13008554
t No.	: 27406222402504	Request Date	: 20/11/2024
lo.	: 4515/24	Verification Date	: 05/12/2024
	: Female	Financial Category	: اورام دار السلام - مبادره
	: 50 Y	Department	: Out Patient
reing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

al Data: Left breast mass
e of Specimen: Tru-cut biopsy.

nique:
tions were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and toxylin as counter stain. The sections treated against:
clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

lts:
(Positive control showed appropriate reactivity):
: Strong nuclear reaction in 90% of tumor cells (score 8/8).
: Strong nuclear reaction in 90% of tumor cells (score 8/8).
r2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
67: 75% nuclear reaction of tumor cells.

clusion:
bt breast mass, Tru-cut biopsy:
rotizing invasive duct carcinoma, Grade III.

R: Positive (score 8/8).
R: Positive (score 8/8).
er2: Positive (score 3+).
i67: Positive in 75% of tumor cells.

coring system: "ER, PR"
SCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

ent Name	الأستاذه / حمديه عبدالرحمن دويب حسن	Request No.	13008554
ent No.	27406222402504	Request Date	20/11/2024
No.	4515/24	Verification Date	05/12/2024
	Female	Financial Category	او رام دار السلام - مبادره
	50 Y	Department	Out Patient
arreing Doc.		Registered by	.marwa mohamed youssa

IMMUNOHISTOCHEMISTRY REPORT

2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and rong). Total score from 0 to 8.
oring system: "Her2"
erence: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining erved or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of nvasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely eptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane ing that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or plete and circumferential membrane staining that is intense and within < 10% of the invasive tumor s. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor s.

Signature
Dr. Rana Fathy
Dr. Menna Nabil

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Hamdeya Abd El Rahman Dowaib Hassan	Date : 28/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 51 Year(s)
Medical ID: 4037/2024	Pat. ID: 94317

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

The last PET CT dated 25-6-2025 was reviewed.

(I) Chest:

- Patent central air ways with well inflated both lungs.
- **Almost complete resolution** of the previously noted right middle lobar and left lingular pulmonary nodules with thin atelectatic bands are noted with tiny one noted at the medial segment of the right middle lobe ,medial segment, measuring 7x3 mm ,being faintly dense in the current study ..
- Scattered areas of air trapping are noted.
- Apart from subcentimetric aortocaval LN, no CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Left breast central density ,for sonomammographic correlation.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density with no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Osteopenic texture of the examined bones with depression of the upper end plate of L4.

OPINION:

- **Follow up status showing the above detailed chnages.**

Dr.Sara Mahmoud, MD

Dr.Sara Ali

Dr.Sara Ali

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة

MDT FORM 1/7/2025 G

Name		MRN	
		ID	
Age		Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	R Pleural
CO morbidity			

Clinical Data

BX: IDC

ER:	PR:	HER 2:	Ki 67:
818	818	+3	

R L

Sonomammography:

Initially presented at 12/2024

Mets Workup: De novo MBC to lungs → 4 cycles taxotere + Herceptin (Phesgo) → CR in lung then received 4 cycles phesgo + Femara Now Progression

TNM c4b1 : lung

Recommendations:

MDT OUTCOME & PLAN

Rec: Tykerb / xeloda

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MRS: HAMDEIA ABDEL RAHMAN DOIB HASSAN
DEAR. PROF. DR. KHELOUD MAHMOUD

PATIENT ID: 1323204
STUDY DATE: 25/06/2025

F18- FDG PET/CT EXAMINATION

History: -

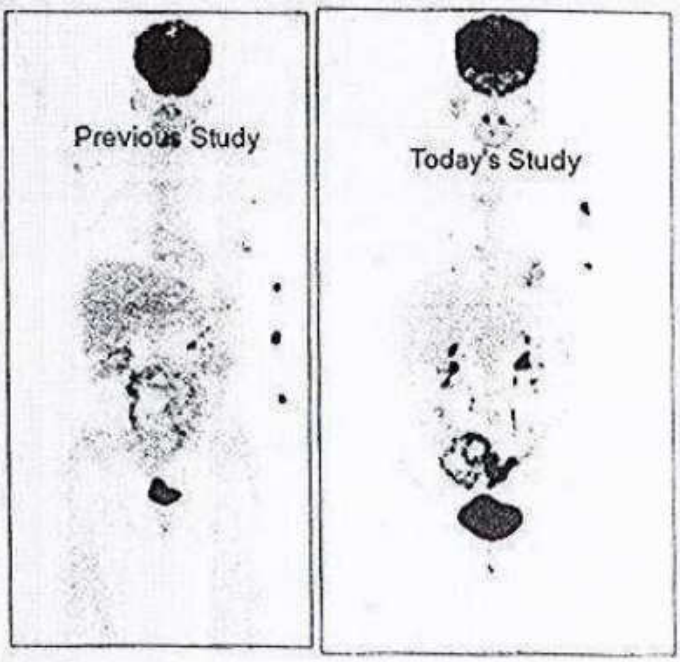
51 years old female patient, with history of metastatic left breast cancer under treatment and referred for follow up PET/CT examination.

Procedure: -

- 8.3 mCi 18F-FDG was administrated to the patient and imaged after appropriate time.
- The blood glucose level was 123 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault to the mid-thigh were obtained.
- A diagnostic 128 MDCT non-contrast examination was taken of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

PET/CT FINDINGS:-

Compared to last PET/CT study dated 05/04/2025 (available DICOM images), the current study revealed:

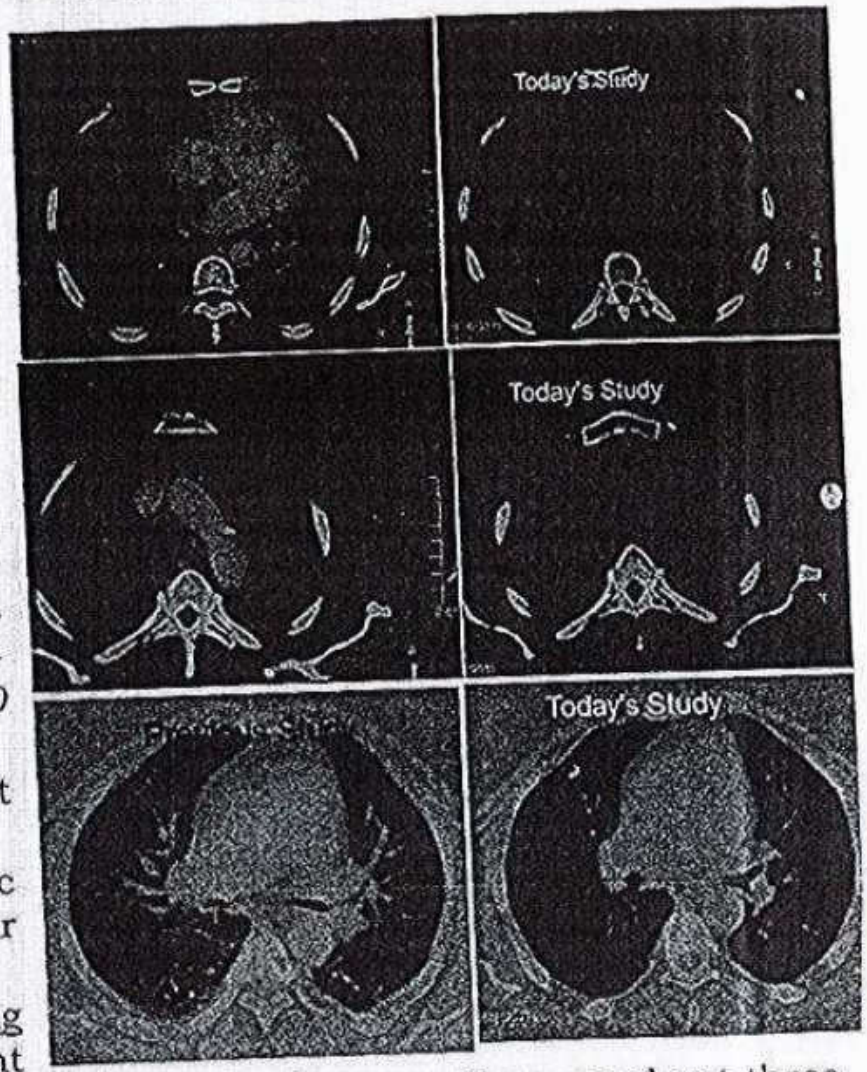


A] Head and Neck: -

- The neck is devoid of any abnormally hypermetabolic lymph nodes. Few small insignificantly avid upper cervical lymph nodes are noted measuring up to 10 mm and achieving 2.69 SUVmax, mostly reactive.
- The brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.
- No FDG avid parenchymal thyroid focal lesions could be elicited.

PET Scan
[[DOCTYPE:PET_SCAN]]

B] Chest:-



- **Metabolic progression** of the left breast mass, currently measuring +/- 42x39 mm and achieving 8.16 SUVmax *(compared to 3.49 SUV max previously)*. Such lesion is inseparable from the overlying skin.
- **Metabolic progression** of the active enlarged left axillary lymph nodes, currently measuring up to +/- 15x12.5 mm and achieving 13.64 SUVmax *(compared to 3.40 SUVmax previously)*
- No hypermetabolic right breast lesions detected.
- No hypermetabolic mediastinal or hilar lymphadenopathy seen.
- Few small mildly avid lung nodules are seen at the right middle lobe and left lingula resting over the fissures. They are about three nodules at the right middle lobe and two nodules at the left lingula. The largest measures 9mm in diameter at the right side and achieves 3.7 SUVmax.
- Normal tracer biodistribution uptake is noted in the myocardium.

PET Scan
[[DOCTYPE:PET_SCAN]]

C] Abdomen & Pelvis:-

- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Physiological uptake in the liver, adrenal glands, spleen and pancreas.
- Physiological excretion of FDG in the kidneys.
- No ascites or hypermetabolic lymphadenopathy noted.

D] Musculoskeletal: -

- No metabolically active osseous lesions detected
- Spondylodegenerative changes of spine are noted.

CONCLUSION:-

Follow up PET/CT study revealed:

- **Metabolic progression of the primary neoplastic left breast lesion and the associated regional left axillary nodal deposits.**
- **Few small mildly avid lung nodules at the right middle lobe and left lingula, mostly metastatic, for follow up.**

MUCH OBLIGED

PROF MOSTAFA ABDELKAWY
PROF DR. OMAR HUSSEIN

HEBA SAMIR, MD

PET Scan
[[DOCTYPE:PET_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Hamdeya Abd El Rahman Dowaib Hassan	Date : 07/12/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 50 Year(s)
Medical ID: 4037/2024	Pat. ID: 94317

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Enlarged in size (about 18 cm at the right MCL), smooth outline, bright echopattern denoting fatty hepatomegaly .*
- *No intra hepatic biliary radicle dilatation.*
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *Average size , normal wall thickness, no stones.*
- *CBD is not dilated.*

Spleen: Average size (About 11 cm), displaying homogenous echopattern.

Kidneys: -

- *Are of normal size and site , normal echogenicity , no stone or backpressure changes , no solid or cystic lesion.*

URINARY BLADDER: Almost empty .

Uterus: -:

- *Not visualized (surgically removed by history).*
- *No gross pelvic lesion .*

Para-aortic and pancreatic area: no gross lesion

Best Regards,
Dr. Sandra fadl

Ultrasound
[[DOCTYPE ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Hamdeya Abd El Rahman Dowaib Hassan	Date : 07/12/2024
Exam : C.T CHEST	Age : 50 Year(s)
Medical ID: 4037/2024	Pat. ID: 94317

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

- **Reported case of left breast cancer.**

I Chest:-

- Left breast retro-areolar spiculated lobulated soft tissue density mass lesion measures about 5x4.8 cm in maximum axial dimensions. It is seen associated with skin thickening and nipple retraction.
- Multiple enlarged left axillary lymph nodes level I, largest measures about 3.5x2 cm.
- Prominent right axillary lymph nodes.
- Multiple diffusely scattered bilateral variable size and shape pulmonary nodules, Largest at anterior segment of right upper lobe measures about 2.1x2.5 cm. It is seen surrounded with multiple thick reticulations.
- No areas of consolidation or cavitary lesions.
- No hilar or mediastinal pathologically enlarged lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Small bilateral elastofibroma dorsi.

II Abdomen & pelvis:

- Mildly enlarged liver with homogenous CT texture. No definte focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Under-filled urinary bladder hindering its proper assessment otherwise no stones.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

Reported case of left breast cancer showing;

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Hamdeya Abd El Rahman Dowaib Hassan	Date : 07/12/2024
Exam : C.T CHEST	Age : 50 Year(s)
Medical ID: 4037/2024	Pat. ID: 94317

- **Left breast spiculated soft tissue mass lesion with pathologically enlarged left axillary lymph nodes as described for sonomammographic correlation.**
- **No evidence of hepatic (non-contrast basis) or osseous metastatic deposits.**

Dr. Shaimaa Khaled, MSc.

Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]